

IMAGES IN CLINICAL MEDICINE

Eczema Herpeticum



A 33-YEAR-OLD PREGNANT WOMAN WITH PREVIOUSLY WELL-CONTROLLED atopic dermatitis presented to the emergency department at 34 weeks' gestation with a 3-day history of fever and an itchy, painful rash. A cold sore had first developed on her lip, after which skin lesions had appeared across her face and body. Physical examination was notable for vesiclopapular lesions on an erythematous base, punched-out erosions (yellow arrow), crusting (black arrow), and scaling on the face and anterior neck (Panel A). Similar lesions were scattered across the chest, arms, and palms. The ocular, oral, and genital mucosa were spared. Polymerase-chain-reaction testing of fluid from a vesicle on the neck was positive for herpes simplex virus type 1 (HSV-1). A serum sample was also positive for HSV-1 IgG antibodies. A diagnosis of eczema herpeticum was made. Eczema herpeticum is a secondary infection of eczematous skin — most often atopic dermatitis — caused by HSV-1. The condition typically occurs on skin affected by the underlying dermatosis. Treatment with intravenous acyclovir was initiated, and the lesions crusted after 72 hours (Panel B). Suppressive oral valacyclovir therapy was continued until the patient had an uncomplicated spontaneous vaginal delivery of a healthy baby at 40 weeks' gestation. At the 5-month follow-up, the skin showed only mild scarring (Panel C).

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